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Exploring the Understanding of Nutrition Facts Tables Among Adults Attending Group Diabetes Education Classes: A Survey of Knowledge, Understanding and Importance of Reading Nutrition Labels Prior to Receiving Any Diabetes Education

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Introduction: Nutrition fact tables (NFTs) were deemed a mandatory component of packaged food products in Canada in 2007. However, past research indicates challenges in patient understanding and use of NFTs. Teaching about healthy eating is a central component of education for patients with Type 2 diabetes (T2DM), but there is a general lack of understanding about NFTs amongst adults attending T2DM groups. There is limited information regarding how nutrition labels are understood and used by people with T2DM. The purpose of the current study is to investigate the use and understanding of NFTs among T2DM group participants. The results will help redesign the teaching of label reading to our T2DM patients.
Methods: A survey (10 questions) was distributed to 70 group participants at the beginning of T2DM class. Forty-four completed surveys were analyzed.
Results: The mean age was 53 (range=28-71); 55% were female. About 2/3 (68%) reported they always or often look at NFTs. Likewise 66% reported that they find label reading useful. Only 36% felt extremely or very confident in their ability to read and understand NFTs, yet 82% felt it is important to learn more about reading NFTs. Age and gender analyses indicated no age differences, but females were significantly more likely to indicate that learning more about NFTs is important ($p<.0001$).
Conclusion: Label reading is clearly an important part of T2DM group education. Our survey respondents reported looking at labels and finding it useful, however this study indicates the need for further education on understanding NFTs.

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What is the Role of Academic Health Centers to Community-based Primary Care Teams in Diabetes Management? Lessons Learned from a Continuing Education Program

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Background: Interprofessional primary care teams such as Family Health Teams (FHTs) in Ontario are expected to play key roles in diabetes education and care. To identify how academic health science centers (AHSCs) can best collaborate with family health teams, an observational case study was conducted exploring their interactions during a two-year, interprofessional continuing education program.
Method: Sequential educational sessions led by a team of interprofessional AHSC facilitators was delivered over two years. Content was case-based, emphasizing group work and team-based learning strategies and approaches to diabetes care. Intended outcomes were the development of new interprofessional clinic models and protocols. Data were collected through interviews, surveys, field notes from educational events and transcripts of communication that took place in between sessions between facilitators and participants.
Results: Roles of AHSC members evolved from that of "content experts" to a role as "mediators" (of discussion or debate within primary care teams) and "mentors" (regarding career development). Nurses, dietitians and social workers sought informal "curbside" clinical advice from AHSC members but family physicians did not. Reasons stemmed from discrepancies between specialist and family physician perceptions of personalization, formality and professionalism.
Conclusions: In the context of diabetes care, there is potential for AHSCs to play a role to primary care teams beyond the traditional role of a "context expert" and "teacher". However cultural differences are significant obstacles to improving collegiality. These should be considered when considering ways to break down silos between the two groups.

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P3 for Public Health: The CANRISK/Pharmacy Collaboration

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Prevention of diabetes and other chronic diseases requires innovative approaches, especially when trying to reach vulnerable populations who may not access health care in traditional settings. Recognizing that pharmacists are trusted health professionals who play an important role in preliminary risk assessment, the Public Health Agency of Canada is collaborating with pharmacies across Canada to raise awareness about diabetes risks.

Pharmacists have been trained to administer the CANRISK diabetes risk questionnaire to help Canadians aged 40-74 identify their risk of pre-diabetes or type 2 diabetes. CANRISK is the only "made in Canada" diabetes risk questionnaire with scientifically-validated weighted scores, and has the potential to improve how we triage people at risk for clinical screening. Currently, there are two Public-Private Partnerships (P3) in place with Shoppers Drug Mart and Pharmasave to support the dissemination of CANRISK, and negotiations are ongoing with other chains.

Having successfully negotiated the Agency's first Public-Private Partnership in chronic disease, this presentation will overview the context and drivers supporting these collaborations, our approach, including successes and challenges, and next steps for evaluating and building on new and existing partnerships. The evaluation will explore the effectiveness of this partnership approach in building public health capacity to undertake diabetes risk assessment and to appropriately triage those at risk to screening and services.

Government, NGO's and the private sector have an increasing role to play in supporting Canadian's health and healthy choices. Stakeholders who are interested in exploring this approach will find this an informative and pragmatic how-to session.

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Bridging Academic and Rural Communities: Lessons Learned from Use of Social Media and Technology in a Distance Learning Program

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Background: Meeting the continuing professional development (CPD) needs of diabetes care providers in rural communities is challenging. To address some of these needs, the Northern Diabetes Health Network engaged the services of the University of Toronto Division of Endocrinology to deliver a learning program.
Method: From February to June 2012, a distance learning program was delivered via videoconferencing to over 200 participants from approximately 40 community-based sites. A specialized multidisciplinary diabetes team from the University of Toronto facilitated the sessions. The program was longitudinal, allowing opportunities to revisit past topics and provide feedback. All sessions emphasized the importance of an interprofessional multidisciplinary team approach to the implementation of care strategies.
Results: The initial didactic style provided little opportunities for interactive learning for the participants. The program was subsequently adjusted to incorporate web-based "pre-tests", social networking, and email to promote information sharing. Participant evaluation reported changes in practice behaviors.
Conclusions: This distance learning program is an example of how the NDHN has leveraged technology to provide healthcare professionals in rural and other areas with accessing professional